# procarbazine, lomustine and vincristine (PCV) chemotherapy · 56-day cycle

**PCV · 56 days** NCCP 00658: adjuvant treatment of grade II glioma after radiotherapy. Use the plan matched to your diagnosis and phase; do not borrow another glioma schedule.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Day 1: lomustine

**Active medicines:** Lomustine

**Emetogenic risk:** Moderate to high (oral)

| Medicine | Dose / route | When |
|---|---|---|
| Oral 5-HT3 antagonist - select ONE | Ondansetron 8-16 mg total daily OR granisetron 1-2 mg total daily; prescriber selects and records agent/dose | Before Day 1 lomustine, then the prescribed delayed-emesis cover<br>NCCP V6 Table 5, p8. Start at least 30 minutes before oral systemic anticancer therapy (SACT) (p4). Confirm stop date, delayed-emesis cover and rescue plan; review QT risk, liver function, constipation and interactions. |

### Additional phase-specific support

- Lomustine and procarbazine have moderate-to-high oral emetic potential. Vincristine alone has minimal emetic risk. The team supplies a day-specific anti-sickness plan, including the procarbazine days, and a rescue plan.
- Vincristine can slow the bowel through its effect on nerves. Preventive laxatives are recommended in NCCP; follow the prescribed plan and report constipation early. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent advice.
- Procarbazine has important interactions. Avoid alcohol during treatment and follow the pharmacist's written food restrictions and advice about when restrictions end. Check all prescription, over-the-counter, herbal and supplement products before taking them.
- Keep prescribed steroids and seizure medicines on a separate plan from chemotherapy. Before vincristine, report new weakness, tingling, walking difficulty or constipation. Check new symptom medicines with the team.
- These are prescription choices for clinician/pharmacist completion, not an issued prescription. General NCCP multiday guidance includes cover after the last SACT day; record the individual duration. Review pre-existing steroid and seizure treatment rather than duplicating it.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____